

1499 [emphasis added].) As alleged, the Court concludes the medical battery claim alleges only professional negligence, such that the statute of limitations of Code of Civil Procedure section 340.5 applies. The claim is untimely as subject to that statute for the reasons set forth above, and the delayed discovery rule does not apply to make the claim timely as explained in connection with the negligence-based claims.

The Medical Center Defendants have asked the Court to review Plaintiff's original complaint in connection with their demurrer to the 2AC in the context of the sham pleading doctrine. Reviewing the original complaint, with statements in the attachment to the original complaint made by Williams under penalty of perjury, Plaintiff alleged that (a) both her husband and her friend Ms. Reed repeatedly informed medical staff treating her that she had severe allergies to medications, (b) a Do Not Give medication list was provided to defendant, and (c) while Plaintiff was incapacitated, her husband in particular objected to the medications Plaintiff was being given and instructed defendants to cease medicating her. (Compl., Exh. A, Exh. 2, pp. 44-49.)

Notably, the allegations that were detailed in the original complaint in this regard are not present in the 2AC. The allegations from the original complaint suggest, however, that Plaintiff may be able to allege facts sufficient to state a medical battery claim based on conditional consent to treat her but not to give her specific medications to which she was allergic, that one or more of the defendants were informed of her allergies and instructed by her family and/or friend not to give specific medications, but that she was nevertheless intentionally administered such medications in violation of their conditional consent. Plaintiff filed her initial complaint within two years after her hospitalization, treatment, and discharge, which would make a medical battery claim timely under Code of Civil Procedure section 335.1. Where there is a reasonable possibility that a complaint may be amended to cure the deficiencies, then leave to amend must be granted. (*Quelimane Co.*, *supra*, 19 Cal.4th at 39.)

The general demurrer to the third cause of action for medical battery is therefore **SUSTAINED, WITH LEAVE TO AMEND**.

4. 9:00 AM CASE NUMBER: C25-02273
CASE NAME: LINDA WILLIAMS VS. SAN RAMON REGIONAL MEDICAL CENTER
HEARING ON DEMURRER TO: 2ND AMENDED COMPLAINT
FILED BY: GROVE, JUDITH, RN
TENTATIVE RULING:

Before the Court is a demurrer by defendants San Ramon Regional Medical Center, Tenet Healthcare Corporation, Judith Grove R.N., and Rene Ochoa, RN to the second amended complaint. For the reasons set forth, the general demurrers to the first, second, and fourth causes of action are **SUSTAINED, WITHOUT LEAVE TO AMEND**; the general demurrer to the third cause of action is **SUSTAINED, WITH LEAVE TO AMEND**.

Background

Plaintiff Linda Williams alleges she was sustained injuries when she was admitted to San Ramon Regional Medical Center on April 20, 2024 and was given medication to which she was allergic.

(Second Amended Complaint ("2AC") ¶¶ 17-25.) She has sued San Ramon Regional Medical Center (the "Medical Center"), Tenet Healthcare Corporation ("Tenet"), two doctors (Drs. Narendra K. Malani and Joseph Toscano), and two nurses (Nurses Judith Grove and Rene Ochoa, each RNs). In her operative 2AC, she alleges causes of action for professional negligence/medical malpractice (1st C/A against all defendants), negligence (2nd C/A against the Medical Center and Tenet), medical battery (3rd C/A against the Medical Center, the doctors, and the nurses), and negligent infliction of emotional distress (NIED, 4th C/A against all defendants).

All defendants have filed demurrers to the 2AC. All defendants other than Dr. Toscano have filed motions to strike portions of the 2AC. Drs. Malani and Toscano have filed joinders in the motion to strike filed by defendants the Medical Center, Tenet Healthcare, RN Grove, and RN Ochoa (collectively the "Medical Center Defendants"). All of the demurrers, motions to strike, and joinders are set for hearing concurrently this date.

Legal Standards Governing Demurrer

In ruling on the demurrer, the Court must accept as true all well-pled factual allegations of the complaint, but not legal or factual conclusions or contentions of law. (*City of Dinuba v. County of Tulare* (2007) 41 Cal. 4th 859, 865; *Carloss v. County of Alameda* (2015) 242 Cal. App.4th 116, 123 [citing *Blank v. Kirwan* (1985) 39 Cal.3d 311, 318].) In determining whether the complaint states a claim for relief, the Court gives "the complaint a reasonable interpretation, reading it as a whole and its parts in their context. [Citation omitted.]" (*Evans v. City of Berkeley* (2006) 38 Cal. 4th 1, 6.) The Court also considers matters of which the Court can properly take judicial notice. (*Carloss, supra*, 242 Cal.App.4th at 123.)

A general demurrer may be sustained when the facts alleged show the action is barred by the statute of limitations. (*Vaca v. Wachovia Mortgage Corp.* (2011) 198 Cal.App.4th 737, 746.) To sustain a demurrer based on the statute of limitations, however, it must "clearly and affirmatively" appear on the face of the complaint that the cause of action is barred, not that it might be barred. (*Committee v. Green Foothills v. Santa Clara County Bd. of Supervisors* (2010) 48 Cal.4th 32, 42; *Lee v. Hanley* (2015) 61 Cal.4th 1225, 1232 [demurrer based on statute of limitations does not lie when the action may be, but is not necessarily, barred].)

Demurrers for uncertainty are disfavored. (*Lickiss v. Financial Industry Regulatory Authority* (2012) 208 Cal.App.4th 1125, 1135.) If a complaint fails to state a cause of action but there is a reasonable possibility of amendment to cure the deficiencies, then leave to amend must be granted. (*Quelimane Co. v. Stewart Title Guaranty Co.* (1998) 19 Cal.4th 26, 39.)

Defendants' Request for Judicial Notice

Defendants ask the Court to take judicial notice of Plaintiff's original complaint, Plaintiff's first amended complaint and the 2AC. The Court grants the unopposed request, subject to the limitations on the scope of judicial notice. (Evid. Code § 452(d); *StorMedia, Inc. v. Superior Court* (1999) 20 Cal.4th 449, 457, fn. 9 [judicial notice does not extend to the truth or proper interpretation of a pleading to the extent reasonably disputable]; *Fremont Indemnity Co. v. Fremont General Corp.* (2007) 148 Cal.App.4th 97, 113-115 [same].)

Notes on Plaintiff's Opposition

PDF page 7 of Plaintiff's opposition to the demurrer is single-spaced, which violates California Rule of Court, Rule 2.108, subdivision (1). The Court has exercised its discretion to consider the opposition in any event.

Plaintiff's opposition also includes a declaration by Plaintiff to which she attaches an "Exhibit A" which she states is a copy of a chronology of her "subsequent medical treatment and selected contemporaneous medical records maintained" by her treating providers. The Court does not consider evidence in ruling on a demurrer except matters subject to judicial notice. (*Carloss, supra*, 242 Cal.App.4th at 123; *Donabedian v. Mercury Ins. Co.* (2004) 116 Cal.App.4th 968, 994.) Though the Court does not consider the declaration and Exhibit to the Williams Declaration to test the sufficiency of the allegations of the 2AC, the Court will consider the declaration as a further offer of proof as to what Plaintiff might be able to allege if the demurrer were sustained and leave to amend granted.

Analysis

The Medical Center Defendants have generally demurred to the 2AC in its entirety and to each of the four causes of action. All of the defendants assert generally similar arguments in their demurrers that Plaintiff's claims are all untimely under Code of Civil Procedure section 340.5 and that her new allegations in the 2AC contradict facts alleged in her original complaint. Nevertheless, the Court makes separate rulings on the demurrers that are tailored as appropriate to the specific defendant's demurrer.

D. Statute of Limitations Applicable to Medical Negligence, Negligence, and Negligent Infliction of Emotional Distress Claims

Code of Civil Procedure 340.5 states in pertinent part, "In an action for injury or death against a health care provider based upon such person's alleged professional negligence, the time for the commencement of action shall be three years after the date of injury or one year after the plaintiff discovers, or through the use of reasonable diligence should have discovered, the injury, whichever occurs first." (Code Civ. Proc. § 340.5.) The statute defines professional negligence generally as "a negligent act or omission to act by a health care provider in the rendering of professional services, which act or omission is the proximate cause of a personal injury or wrongful death." (Code Civ. Proc. § 340.5 subpart (2).)

Code of Civil Procedure section 364 requires a party to serve a Notice of Intent before commencing an action for medical negligence. (Code Civ. Proc. § 364(a) ["No action based upon the health care provider's professional negligence may be commenced unless the defendant has been given at least 90 days' prior notice of the intention to commence the action."].) The statute also provides, "If the notice is served within 90 days of the expiration of the applicable statute of limitations, the time for the commencement of the action shall be extended 90 days from the service of the notice." (Code Civ. Proc. § 364(d).)

Plaintiff alleges causes of action for professional negligence/medical malpractice (1st C/A), negligence (2nd C/A), and negligent infliction of emotional distress (4th C/A) all arising out of allegations regarding the rendition of medical treatment and related services by defendants. There appears to be no dispute among the parties that Code of Civil Procedure section 340.5 applies to those three causes of action.

The elements of negligence, including professional negligence or medical malpractice, are duty, breach of duty and causation. (*Ladd v. County of San Mateo* (1996) 12 Cal.4th 913, 917.) (*See also Flowers v. Torrance Memorial Hospital Medical Center* (1994) 8 Cal.4th 992, 995, 997-998 ["[W]e consider the distinction between 'ordinary' and 'professional' negligence and conclude that with respect to questions of substantive law they comprise essentially one form of action. Apart from statutory considerations, characterizing misfeasance as one type of negligence or the other generally only serves to define the standard of care applicable to the defendant's conduct."].)

Negligent infliction of emotional distress is not a tort separate from the tort of negligence. "We have repeatedly recognized that '[t]he negligent causing of emotional distress is not an independent tort, but the tort of negligence. [Citation.] The traditional elements of duty, breach of duty, causation, and damages apply. [¶] Whether a defendant owes a duty of care is a question of law. Its existence depends upon the foreseeability of the risk and a weighing of policy considerations for and against imposition of liability. [Citation.].' [Citations omitted.]" (*Burgess v. Superior Court* (1992) 2 Cal.4th 1064, 1072.) (*See also Marlene F. v. Affiliated Psychiatric Medical Clinic, Inc.* (1989) 48 Cal.3d 583, 588.) If a negligence cause of action is established, then damages for severe emotional distress may be recoverable based on the breach of duty owed to the plaintiff by the defendant. (*Marlene F., supra*, 48 Cal.3d at 590[.]) (*See also Potter v. Firestone Tire & Rubber Co.* (1993) 6 Cal.4th 965, 984 ("Potter") [under California law, damages for emotional distress are recoverable only if the defendant has breached some other duty to the plaintiff].)

The Medical Center Defendants argue as a basis for the demurrer that Plaintiff's negligence claims are duplicative. The Court does not need to reach that argument as a basis for determining the demurrers to those causes of action, given the Court's conclusions regarding the statute of limitations applicable to the negligence-based claims.

E. Accrual of Statute of Limitations, Delayed Discovery Allegations, and Sham Pleading Doctrine Applied to Negligence Causes of Action

Plaintiff alleges after her admission to the Medical Center on April 20, 2024, she was given medication to which she was allergic and had a severe allergic reaction during her hospitalization. (2AC ¶¶ 1-3, 17-25.) Based on her allegations, all the essential elements of her cause of action were complete during her hospitalization, and unless tolled, the statute of limitations accrued when she sustained her injuries caused by the alleged negligent treatment. Plaintiff relies on the delayed discovery rule to toll the accrual of her claims and to make her initial complaint timely.

1. Delayed Discovery Rule

" 'Generally speaking, a cause of action accrues at 'the time when the cause of action is complete with all of its elements.' [Citations omitted.] An important exception to the general rule of accrual is the 'discovery rule,' which postpones accrual of a cause of action until the plaintiff discovers, or has reason to discover, the cause of action. [Citations omitted.]" (*Fox v. Ethicon Endo-Surgery, Inc.* (2005) 35 Cal.4th 797, 806-807.) "[U]nder the delayed discovery rule, a cause of action accrues and the statute of limitations begins to run when the plaintiff has reason to suspect an injury and some wrongful cause, unless the plaintiff pleads and proves that a reasonable investigation at that time would not have revealed a factual basis for that particular cause of action. In that case, the statute of limitations for that cause of action will be tolled until such time as a reasonable investigation would

have revealed its factual basis." (*Id.* at 803.)

"In order to rely on the discovery rule for delayed accrual of a cause of action, '[a] plaintiff whose complaint shows on its face that his claim would be barred without the benefit of the discovery rule must specifically plead facts to show (1) the time and manner of discovery and (2) the inability to have made earlier discovery despite reasonable diligence.' [Citation omitted.]" In assessing the sufficiency of the allegations of delayed discovery, the court places the burden on the plaintiff to 'show diligence'; 'conclusory allegations will not withstand demurrer.' [Citation omitted.] [¶] [T]o employ the discovery rule to delay accrual of a cause of action, a potential plaintiff who suspects that an injury has been wrongfully caused must conduct a reasonable investigation of all potential causes of that injury. If such an investigation would have disclosed a factual basis for a cause of action, the statute of limitations begins to run on that cause of action when the investigation would have brought such information to light. In order to adequately allege facts supporting a theory of delayed discovery, the plaintiff must plead that, despite diligent investigation of the circumstances of the injury, he or she could not have reasonably discovered facts supporting the cause of action within the applicable statute of limitations period." (*Id.* at 808-809.)

"A plaintiff need not be aware of the specific 'facts' necessary to establish the claim; that is a process contemplated by pretrial discovery. Once the plaintiff has a suspicion of wrongdoing, and therefore an incentive to sue, she must decide whether to file suit or sit on her rights. So long as a suspicion exists, it is clear that the plaintiff must go find the facts; she cannot wait for the facts to find her." (*Jolly v. Eli Lilly & Co.* (1988) 44 Cal. 3d 1103, 1111.) Rather, " 'when the patient's "reasonably founded suspicions [have been aroused]," and she has actually "become alerted to the necessity for investigation and pursuit of her remedies," the one-year period for suit [under Code of Civil Procedure section 340.5] begins.' [Citation omitted.]" (*Warren v. Schechter* (1997) 57 Cal. App. 4th 1189, 1201.)

2. Allegations Regarding Delayed Discovery in the 2AC

Plaintiff alleges she served Notices of Intent to sue on April 16, 2025. The complaint initiating this action was filed on August 7, 2025, more than 90 days after the notices were served and outside the 90-day tolling period provided in Code of Civil Procedure section 364. Unless the statute of limitations did not begin to run until on or after August 7, 2024, Plaintiff's complaint commencing this action is barred by the statute of limitations.

Plaintiff repeatedly alleges that during her hospitalization, which commenced April 20, 2024, she was medically compromised, unconscious, sedated, or experiencing a serious medical event. (2AC ¶¶ 33-35, 37.) The Medical Center Defendants highlight that these allegations alone demonstrate her knowledge, or at least suspicion, that she had suffered injury beginning on April 20, 2024 after she was admitted to the hospital so as to start the statute of limitations running.

In the 2AC, Plaintiff alleges that she "first began to suspect" that "her injuries were connected to wrongful hospital care" after "observing unexplained bruising, experiencing continuing symptoms, reviewing vague discharge-related information, and consulting with treating medical providers in the months following her return home." (2AC ¶ 38.) She implicitly concedes she had some hospital records when she was discharged, but she alleges she did not have complete records (she describes 573 pages of records being produced in 2026, 2AC ¶¶ 44, 45), and she could not "meaningfully

compare medication orders, medication administration records, allergy or warning information, discharge information, restraint-related information, and later produced records" until she reviewed later produced records. (2AC ¶¶ 36, 38, 39, 40.) She alleges she learned "additional restraint-related facts from Ms. Reed" who was at the hospital during "nearly all available visiting hours" and who "believed Plaintiff already knew those facts." (2AC ¶ 46 [emphasis added].) Plaintiff alleges the discovery materials and "later-learned restraint facts" "confirmed, clarified and expanded facts Plaintiff had previously suspected." (2AC ¶ 47 [emphasis added].) (See also 2AC ¶ 48 [alleging she "gradually discovered and reasonably suspected wrongful hospital care and medication related causation only after discharge, after observing unexplained bruising and continuing symptoms, reviewing vague and incomplete discharge-related information," among other things [emphasis added].)

Plaintiff has clearly alleged she was fully aware she had suffered a serious medical event during her hospitalization. (2AC ¶¶ 33-35, 37.) Plaintiff does not allege that after she was discharged, she was unable to make inquiries of her husband and friend Ms. Reed, who was at the hospital during almost all visiting hours, regarding what occurred during her hospitalization. She does not allege she was not given any discharge-related paperwork when she was discharged from the hospital on April 22, 2024. She does not allege that any bruising or symptoms did not appear until months after her discharge such that she did not suspect injury or a wrongful cause until months later. She therefore does not allege facts showing that "despite diligent investigation of the injury" she could not have discovered facts supporting the wrongful cause within the applicable statute of limitations. Indeed, she clearly knew the facts by April 16, 2025 when she served her Code of Civil Procedure section 364 notices.

Paragraph 38 of the 2AC states Plaintiff "first began to suspect" her injuries "were connected to a wrongful" cause (wrongful hospital care) "after observing unexplained bruising, experiencing symptoms, reviewing vague discharge-related information, and consulting with treating medical providers in the months following her return home." (2AC ¶ 38.) The bruising, other symptoms, and "discharge-related information" must reasonably have been available to Plaintiff at or shortly after her discharge, such that based on these allegations, she began to suspect injury and a wrongful cause shortly after her discharge (*Fox, supra*, 35 Cal.4th at 803.)

Plaintiff does not allege specific facts showing when and how she discovered she was injured and was given improper medication, alleging only in the most general terms that her knowledge developed over some unspecified time period through her general observations of bruising, review of discharge-related information and consulting with physicians over an indeterminate time period. (*Fox, supra*, 35 Cal.4th at 808.) She also does not allege specific facts showing that she could not have discovered with reasonable diligence, such as through communications with her husband and Ms. Reed, sufficient information to inform her that she received improper medication and was restrained at or shortly after she was released from the hospital. (*Id.*)

The allegations of the 2AC are insufficient to allege delayed discovery of her negligence-related claims under the above-cited standards so as to make the complaint timely filed within the statute of limitations. The case law provides that the cause of action accrues and the statute of limitations commences to run under the delayed discovery rule when the plaintiff first suspects injury and a wrongful cause, not when the plaintiff has complete evidence documenting witnesses and the wrongful acts causing the injury. (See, e.g., 2AC ¶¶ 44, 45.) Suspicion of injury and a wrongful cause,

not knowledge of all witnesses, complete records, and a full investigation of all facts supporting her negligence-related claims, is all that is required for the statute of limitations of Code of Civil Procedure section 340.5 to begin to run under *Fox*.

3. Application of the Sham Pleading Doctrine Based on Original Complaint

Even if the allegations of the 2AC were sufficient taken alone to allege delayed discovery to some vague time frame on or after August 7, 2024, the Medical Center Defendants also rely on the "sham pleading" doctrine, arguing that Plaintiff has made factual allegations in the 2AC that contradict or are inconsistent with her allegations in the original complaint to try to avoid the claims being barred by the statute of limitations. (*Larson v. UHS of Rancho Palm Springs* (2014) 230 Cal.App.4th 336, 343-344.) Related to the sham pleading doctrine is the "truthful pleading" doctrine. (*Cantu v. Resolution Trust Corp.* (1992) 4 Cal.App.4th 857, 877-878 [doctrine of "truthful pleading" precludes a party from avoiding a demurrer by suppressing facts that prove the pleading false].) (*See also Hendy v. Losse* (1991) 54 Cal.3d 723, 742-743 [explaining doctrines, stating they apply to both verified and unverified complaints, and affirming order sustaining demurrer without leave to amend where plaintiff alleged initially on information and belief that defendant was an employee and sought to amend to allege defendant was an independent contractor without demonstrating original allegation was "the result of inadvertence or mistake, or that he has since discovered a factual basis for alleging" defendant was an independent contractor]; *Deveny v. Entropin, Inc.* (2006) 139 Cal.App.4th 408, 425-427 [stating, among other things, "Under the sham pleading doctrine, plaintiffs are precluded from amending complaints to omit harmful allegations, without explanation, from previous complaints to avoid attacks raised in demurrers or motions for summary judgment. [Citation omitted.]" (Emphasis added.)].)

" 'Generally, after an amended pleading has been filed, courts will disregard the original pleading. [Citation.] [¶] However, an exception to this rule is found . . . where an amended complaint attempts to avoid defects set forth in a prior complaint by ignoring them. The court may examine the prior complaint to ascertain whether the amended complaint is merely a sham.' [Citation.] . . . Moreover, any inconsistencies with prior pleadings must be explained; if the pleader fails to do so, the court may disregard the inconsistent allegations. [Citation.] Accordingly, a court is "not bound to accept as true allegations contrary to factual allegations in former pleading in the same case." [Citation.]" [Citations omitted.]" (*Larson, supra*, 139 Cal.App.4th at 343-344.) (*See also Nolte v. Cedars-Sinai Medical Center* (2015) 236 Cal.App.4th 1401, 1406 [" 'A plaintiff may not avoid a demurrer by pleading facts or positions in an amended complaint that contradict the facts pleaded in the original complaint or by suppressing facts which prove the pleaded facts false.' [Citation omitted.]"].)

The Court agrees that the truthful pleading/sham pleading doctrines apply here. Plaintiff has notably omitted harmful allegations made in her original complaint (Compl. Exh. A, pp. 42-43), including the date she was discharged (April 22, 2024), the fact that she was conscious and aware as of midday on April 22, 2024 when she was discharged, that she observed bruising on her wrists and arms when she was discharged and photographed the bruising when she returned to Tennessee the next day, and that she experienced the symptoms of an allergic reaction to acetaminophen for several days after she returned home. (Compl. Exh. A ¶ 40, p. 24, Exh. 2 pp. 43-49.) Plaintiff does not explain or justify her omission of those facts alleged in her original complaint when she filed the 2AC. (*Deveny, supra*,

139 Cal.App.4th at 426.)

Plaintiff's current allegations of delayed discovery in her 2AC are contradicted by, and inconsistent with, her factual allegations in her original complaint regarding when she had information sufficient for her to suspect she had been injured and that the injury was the result of a wrongful cause, namely, administration of inappropriate medications. For example, in the 2AC she alleges she began to suspect "wrongful hospital care after observing unexplained bruising" and suggests at the end of the paragraph her suspicion arose "in the months following her return home." (2AC ¶ 38.) In her original complaint, she alleged her first memory of her hospitalization was midday on Monday, April 22, 2024, and she alleged she was on a FaceTime call with her husband when a nurse came in to stop the blood pressure medication she was being administered. (Compl. Exh. A, Exh. 2 pp. 48-49.) She alleged, "When I left the hospital I had multiple IV sites and multiple unexplained bruises. I had severe allergic reaction symptoms from the Acetaminophen for several days after leaving the hospital making it impossible for me to eat solid food (I had pictures taken again of the trauma as soon as I returned back to TN the next day) **See Exhibit 3.**" (Compl. Exh. A, Exh. 2 p. 49 [emphasis added].) (See also Compl., Exh. A ¶ 40, p. 24 [left hospital with unexplained bruises].)

In the 2AC she alleges she was consulting with her medical providers during the months after she returned home, but she alleges she did not have her "complete chart" and other medical records at the time of her hospitalization. (2AC ¶¶ 36, 38.) In her original complaint, she provides details that undermine those allegations, stating her treatment for "PTSD" and other symptoms from the alleged improper medical care at the hospital from April 20-22, 2024 "is being coordinated by my primary physician who was outraged when she read the 92-page Hospital report." (Compl. Exh. A, Exh. 2 p. 49 [emphasis added].) Plaintiff in the 2AC alleges that she received the 573-page production of medical records in 2026 (2AC ¶¶ 44, 45) and that the "original discharge paperwork available to her did not disclose or meaningfully identify Mr. Williams or Ms. Reed as material participants or witnesses." (2AC ¶ 45.) However, Plaintiff specifically alleged in her original complaint she was told what occurred during her hospitalization by her husband, her friend Heather [Reed], her friend's sister Merrie Reed, and the 92-page report from San Ramon Regional Medical Center, a copy of which appears to be attached to her original complaint. (Compl. Exh. A, Exh. 2 p. 45, and pp. 93-187.) Indeed, her original complaint shows she filed complaints with the medical board about her treatment on June 10, 2024. (Compl. pp. 190-195.)

Plaintiff alleges no specific facts showing that beginning promptly after her discharge and return to Tennessee, when she observed bruising on her arms and wrists according to her original complaint, she was unable to investigate information regarding the events of her hospitalization through discussions with Mr. Williams (her husband) or Ms. Reed (her friend who was at the hospital during almost all visiting hours). (Compare 2AC ¶¶ 45, 46 to Compl. Exh. A, Exh. 2 pp. 44-49 [describing at length both her husband and Ms. Reed telling hospital and staff not to administer medications without approval of one of them, repeated instructions by husband and Ms. Reed to stop giving Plaintiff medication and giving the defendants a list of medications not to give, and alleging that her husband and Ms. Reed told her what happened during her hospitalization].) To the contrary, her prior complaint shows that she was informed of what happened during her hospitalization by her husband and friend and that the 92-page hospital report had information sufficient for her physician to be "outraged" by her treatment. She does not allege any facts asserting that she could not, or did not,

immediately investigate with her husband and friend the events that occurred during her hospitalization, particularly when in her original complaint she admits she was aware of bruising on her arms and the after effects of her acetaminophen allergic reaction when she was discharged.

Though the Williams Declaration and the attached Exhibit A is evidence outside the 2AC and cannot make the 2AC sufficient if its allegations fail, Williams asks for leave to amend in her opposition if the demurrer were sustained. The Court has considered Exhibit A to her declaration for that purpose. She argues that the excerpts of the medical records show an "increasing understanding" and "realization." Instead, they show that as of no later than May 13, 2024, she had sufficient suspicion of her injury and its wrongful cause for the statute of limitations to have accrued. The Court finds the exhibit reinforces Plaintiff's knowledge of her injury and the wrongful cause shortly after her discharge. Specifically, a note in the excerpts of medical records show that as of May 13, 2024, Plaintiff had informed her doctor that she was treated with propofol in the hospital to which she is allergic and that she suspected she was restrained in the hospital based on bruising on her arms and wrist.

(Williams Decl. re Opp. Exh. A, entry dated 5/13/2024 PDF pp. 17-18.) These facts further support her knowledge of the factual basis of her claims by no later than May 2024 and undermine her vague allegations that her discovery of her claim developed over time, and some time on or after August 7, 2024. Again, these facts are considered only on the issue of the ability to amend to avoid the bar of the statute of limitations.

4. Conclusion as to Statute of Limitations on Three Negligence-Based Causes of Action

The facts alleged in the 2AC regarding delayed discovery are insufficient to state specific facts showing that she did not suspect her injuries and their wrongful cause until months after her discharge under the requirements of *Fox*. Further, Defendant has shown that the factual allegations of the 2AC regarding when she discovered or suspected her injury and its wrongful cause are inconsistent with and contradicted by facts alleged in her original complaint which were set forth under penalty of perjury, such that the sham pleading doctrine applies. Plaintiff has not demonstrated that she can allege facts to truthfully plead a claim for delayed discovery that would save her negligence-based causes of action from being barred by the statute of limitations of Code of Civil Procedure section 340.5, since her own evidence arguably offered to support allowing leave to amend shows that she had knowledge or at least suspicion of her injuries and their wrongful cause as of May 13, 2024. The complaint initiating the action was filed more than one year after that date, and more than 90 days after she served her Notices of Intent to Sue under Code of Civil Procedure section 364.

The claims in the first, second, and fourth causes of action are barred by the statute of limitations of Code of Civil Procedure section 340.5. The general demurrers to the first, second, and fourth causes of action are **SUSTAINED, WITHOUT LEAVE TO AMEND.**

5. Medical Battery Claim (3rd C/A)

Plaintiff also alleges a claim for medical battery (3rd C/A). Battery, and specifically medical battery, is an intentional tort. (*Piedra v. Dugan* (2004) 123 Cal. App. 4th 1483, 1499.) For a claim of battery, a two-year statute of limitations applies under Code of Civil Procedure section 335.1. (Code Civ. Proc. § 335.1 ["Within two years: An action for assault, battery, or injury to, or for the death of, an individual caused by the wrongful act or neglect of another."]; *Samantha B. v. Aurora Vista Del Mar, LLC* (2022)

77 Cal.App.5th 85, 96; *Benun v. Superior Court* (2004) 123 Cal.App.4th 113, 126.)

"The essential elements of a cause of action for battery are: (1) defendant touched plaintiff, or caused plaintiff to be touched, with the intent to harm or offend plaintiff; (2) plaintiff did not consent to the touching; (3) plaintiff was harmed or offended by defendant's conduct; and (4) a reasonable person in plaintiff's position would have been offended by the touching." (*So v. Shin* (2013) 212 Cal.App.4th 652, 669.) "A medical battery occurs where 'a doctor obtains consent of the patient to perform one type of treatment and subsequently performs a substantially different treatment for which consent was not obtained.' " (*Id.*)

The Medical Center Defendants contend the medical battery cause of action is uncertain, and the allegations are not specific or sufficient to allege medical battery based on lack of consent. They argue that "administration of medications simply because Plaintiff opposed their use does not constitute a medical battery," an argument the Court finds somewhat baffling as a battery is by definition a touching or administration of a procedure or treatment to which the plaintiff did not consent. (MPA ISO Dem. p. 9, ll. 11-12.) They argue there is a "dispute" as to whether she had capacity to consent given her condition when she arrived at the hospital and whether she was fully informed of the risks, and therefore her claim is one for negligence. (MPA ISO Dem. p. 9, ll. 12-16.) These arguments are not fully consistent with the allegations of the 2AC and with the facts alleged in the original complaint. They do not address what the Court perceives as the essence of Plaintiff's medical battery cause of action, though for the reasons explained below, the Court agrees the cause of action as presently alleged fails to state sufficient facts to allege the medical battery cause of action.

The Court interprets Plaintiff's allegations as attempting to allege medical battery based on conditional consent. (2AC ¶¶ 68-76.) " 'There are three elements to a claim for medical battery under a violation of conditional consent: the patient must show his consent was conditional; the doctor intentionally violated the condition while providing treatment; and the patient suffered harm as a result of the doctor's violation of the condition. [Citation.]' [Citation omitted.]" (*Piedra v. Dugan* (2004) 123 Cal. App. 4th 1483, 1497-1498 [quoting *Conte v. Girard Orthopaedic Surgeons Medical Group, Inc.* (2003) 107 Cal.App.4th 1260, 1269].) "[A] claim for battery against a doctor as a violation of conditional consent requires proof the doctor intentionally violated the condition placed on the patient's consent. [Citation omitted.]" (*Id.* at 1499 [emphasis added].)

Notwithstanding the title of the cause of action as "medical battery," the determination of whether the one-year statute of limitations of Code of Civil Procedure section 340.5 for medical negligence applies depends on the gravamen of the cause of action. "[W]hen a plaintiff asserts a claim against a health care provider on a legal theory other than professional negligence, courts must determine whether the claim is nonetheless based on the health care provider's professional negligence, which would require application of MICRA. [Citations omitted.].) To make that determination, courts must examine not only the legal theory alleged, but also the nature of the health care provider's alleged conduct and the legislative history of the MICRA provision at issue. [Citations omitted.] When . . . the question presented concerns which limitations period applies, courts also must focus on the nature or gravamen of the claim, not the label or form of action the plaintiff selects. [Citations omitted.]" (*Larson, supra*, 230 Cal.App.4th at 347.)

In *Larson, supra*, 230 Cal.App.4th 336, the Court held that the statute of limitations of Code of Civil

Procedure section 340.5 applied to the plaintiff's medical battery claim. The Court explained, "Although Larson labels his claims as intentional torts for battery and intentional infliction of emotional distress, the operative complaint's factual allegations, when read in conjunction with Larson's complaints in the Earlier Action, reveal Larson's claims are based on professional negligence within the meaning of section 340.5. Indeed, Larson alleges Shuman was the anesthesiologist for his surgery and injured Larson by forcefully grabbing and twisting his arm while conducting a preoperative checkup, and by prying open Larson's mouth and violently punching, lifting, and pushing Larson's face as he put on the mask to administer anesthesia. These allegations challenge the manner in which Shuman rendered the professional health care services he was hired to perform; they do not allege intentional torts committed for an ulterior purpose." (*Id.* at 351 [emphasis added].)

Plaintiff alleges that "to the extent any consent was obtained" from her or her family or authorized persons, it was provided "if at all" and limited to medically appropriate medications, and that there was no consent to "the administration of medications that were contraindicated, restricted, documented as unsafe for Plaintiff, or otherwise contrary to medication warnings, allergy-related information, adverse reaction history, contraindications, and 'Do Not Give' restrictions known or reasonable knowable to Defendants." (2AC ¶ 70.) (*See also* ¶¶ 18 ["Defendants had access to information" concerning Plaintiff's medication allergies], 19 ["medication-related warnings and restrictions were communicated" and medication restrictions were "known or knowable"], 25 ["Defendants' acts and omissions were not isolated clerical errors, but involved failures in medical decision-making, nursing care, communication, chart review, medication administration, patient safety, and hospital systems." (emphasis added)], 69, 71-73.) She alleges that the Defendants "intentionally performed, authorized, ordered, directed, permitted, administered, and/or caused the administration of medication related treatment to Plaintiff." (2AC ¶¶ 70, 72.)

She does not allege that the Medical Center Defendants "intentionally violated the condition placed on the patient's consent" which is required to state a claim for medical battery. (*Piedra, supra*, 123 Cal.App.4th at 1499 [emphasis added].) (*See also Ashcraft v. King* (1991) 228 Cal.App.3d 604, 610-613 [cited by the Medical Center Defendants, in which the Court reversed a nonsuit for defendant on a medical battery claim where evidence showed plaintiff consented to a blood transfusion only from family members but was given blood from nonfamily donors and contracted HIV].) As alleged, the Court concludes the medical battery claim alleges only professional negligence, such that the statute of limitations of Code of Civil Procedure section 340.5 applies. The claim is untimely as subject to that statute for the reasons set forth above, and the delayed discovery rule does not apply to make the claim timely as explained in connection with the negligence-based claims.

The Medical Center Defendants have asked the Court to review Plaintiff's original complaint in connection with the demurrer to the 2AC in the context of the sham pleading doctrine. Reviewing the original complaint, with statements in the attachment to the original complaint, Plaintiff alleged that (a) both her husband and her friend Ms. Reed repeatedly informed medical staff treating her that she had severe allergies to medications, (b) a Do Not Give medication list was provided to defendants, and (c) while Plaintiff was incapacitated, her husband in particular objected to the medications Plaintiff was being given and instructed defendants to cease medicating her. (Compl., Exh. A, Exh. 2, pp. 44-49.)

Notably, the allegations in the original complaint in this regard are not present in the 2AC. The

allegations from the original complaint suggest, however, that Plaintiff may be able to allege facts sufficient to state a medical battery claim based on conditional consent to treat her but not to give her specific medications to which she was allergic, that one or more of the defendants were informed of her allergies and instructed by her family and/or friend not to give specific medications, but that she was nevertheless intentionally administered such medications in violation of their conditional consent. Plaintiff filed her initial complaint within two years after her hospitalization, treatment, and discharge, which would make a medical battery claim timely under Code of Civil Procedure section 335.1. Where there is a reasonable possibility that a complaint may be amended to cure the deficiencies, then leave to amend must be granted. (*Quelimane Co.*, *supra*, 19 Cal.4th at 39.)

The general demurrer to the third cause of action for medical battery is therefore **SUSTAINED, WITH LEAVE TO AMEND.**

5. 9:00 AM CASE NUMBER: C25-02273
CASE NAME: LINDA WILLIAMS VS. SAN RAMON REGIONAL MEDICAL CENTER
*HEARING ON MOTION IN RE: STRIKE 2ND AMENDED COMPLAINT
FILED BY: GROVE, JUDITH, RN
TENTATIVE RULING:

Before the Court is a motion by defendants San Ramon Regional Medical Center, Tenet Healthcare Corporation, Judith Grove R.N., and Rene Ochoa, R.N. to strike portions of the second amended complaint. Defendants Narendra K. Malani, M.D. and Joseph Toscano, M.D. have joined in the motion. The motion and joinders are **DENIED AS MOOT**, based on the Court's concurrent ruling on the demurrers to the second amended complaint by the moving and joining defendants.

6. 9:00 AM CASE NUMBER: C25-02273
CASE NAME: LINDA WILLIAMS VS. SAN RAMON REGIONAL MEDICAL CENTER
*HEARING ON MOTION IN RE: STRIKE PLTFS 2ND AMENDED COMPLAINT
FILED BY: MALANI, DR. NARENDRA K
TENTATIVE RULING:

Before the Court is a motion by defendant Narendra K. Malani, M.D. to strike portions of the second amended complaint. The motion is **DENIED AS MOOT**, based on the Court's concurrent ruling on the demurrer to the second amended complaint by the moving defendant.

7. 9:00 AM CASE NUMBER: C25-02273
CASE NAME: LINDA WILLIAMS VS. SAN RAMON REGIONAL MEDICAL CENTER
HEARING IN RE: JOINDER IN CO-DEF SAN RAMON REGIONAL MEDICAL CENTER, ET AL.'S MOTION TO STRIKE PLAINTIFF'S SECOND AMENDED COMPLAINT
FILED BY: TOSCANO, DR. JOSEPH
TENTATIVE RULING:

See Line 5 above.